



جوستاف روسي انترناشيونال - مصر (EPE) * طلب علاج أول مرة / متردد

إسم المريض ربيعاً : فاطمة خليفة زهران عبدالمولى

العنوان : ق

الحى / المدينة : الهرم

المحافظة : الجيزة

تاريخ تسجيل الإستمارة : 03-10-2026

رقم البطاقة : 28001102104244 رقم التليفون : 01145485381

العمر (تاريخ الميلاد) : 10-01-1980 المهنة : ربه منزل

الإقرار

أقر أنا الموقع أدناه انني لا اتمتع باي نظام علاج مثل :-التأمين الصحي-القوات المسلحة-الشرطة-الهيئات-المؤسسات او اي نظام يتعارض مع نظام العلاج علي نفقة الدولة- وسوف اتحمل كافة نفقات العلاج في حالة ثبوت غير ذلك

الإخصائي الاجتماعي
التوقيع: (-----)

المقرر بما فيه
التوقيع: (-----)

التشخيص الإكلينيكي : سرطان بالثدي (مبادرة)

نتائج الأبحاث : (مرفق)

التشخيص النهائي : سرطان بالثدي (مبادرة)

خطة العلاج : أبحاث أورام مزيد من الابحاث و الاشعات [7954] قسم العلاج : الاورام

التعليق على التشخيص : -----

تقرير اللجنة الثلاثية المتخصصة

الاسم: د/محمد كيلاني
الاسم: د/ندى حسن
الاسم: د/نرمين رمضان

الوظيفة: -----
الوظيفة: -----
الوظيفة: -----

التوقيع: -----
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أطلعت اللجنة علي لائحة العلاج الخاصة بالمجالس وتلتزم جهة العلاج بلائحة المجالس الطبية. كما تقرر اللجنة بان البيانات المدونة فى هذا النموذج صحيحة.

تعتمد صحة البيانات المرفقة
رئيس اللجنة ومدير
المستشفى.

تعهد المستشفى أن لا يتم تنفيذ القرار إلا بعد مراجعة الملف الطبي للمريض وإعادة مناظرة المريض بالمستشفى قبل الصرف.

أعضاء اللجنة

د. / ----- د. / ----- د. / ----- د. / -----

قرار اللجنة

التشخيص النهائي : -----
خطة العلاج : -----
المستشفى : -----
جهة العلاج : -----
ملاحظات : -----

قسم : -----
نفقات : -----

ختم
شعار الجمهورية

مدير عام
مديرية الشؤون الصحية

رئيس
المجلس الطبي المتخصص

--

Name: فاطمة خليفة زهران عبدالمراد National ID: 28001102104244	
--	---

تقرير طبي

السيد
تحية طبية وبعد,,

إيماء للطلب المقدم من	بشأن تقرير طبي للمريض
بقسم:	رقم طبي:

نتشرف بان نوضح فيما يلي تقريرنا عن حالته
--

تاريخ وساعة الدخول : -- --:-- yyyy-mm-dd	تاريخ وساعة الخروج : PM 01:17 05-07-2026
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وبعد الكشف والفحوصات والمتابعة وجد انه
--

المریضة تعاني من ورم غير حمید بالثدي و تحتاج الي Further investigations and imaging

التاريخ : 05-07-2026	الوقت : -- --:--
	الطبيب المعالج : د/شيماء لاشين

جمهورية مصر العربية
علاقة تحمل الشخصية



فاطمة

خليفة زهران عبدالمولى

١٢ ش عبدالنواب البهنساوى - العرويه
الهرم - الجيزه

١٩٨٠/٠١

٢٨٠٠١١٠٢١٠٤٢٤٤

LB8364962



٢٠٢٥/٠٤

٢٨٠٠١١٠٢١٠٤٢٤٤

ربه منزل

متزوجة

مسلمة

أنثى

احمد حمدي محمود ابراهيم

البطاقة سارية حتى ٢٠٢٢/٠٤/٠٥



**Prof. Dr.
Essam Ayad**

Professor of Pathology, Cairo University

Consultant Pathologist.

Italian, El Nada, Al-Salam & Neuro Espitalia Hospitals

Member of the Society of Pediatric Pathology, USA

Fellow of Padova University- Italy

دكتور
عصام عياد

أستاذ الباثولوجى كلية الطب - جامعة القاهرة

استشاري الباثولوجى

بمستشفيات الايطالى والندى والسلام ونيوروسبيتاليا

عضو جمعية باثولوجى الأطفال بأمريكا

زميل جامعة بادوفا بايطاليا

PATHOLOGY REPORT

Patient Name: فاطمه خليفه زهران عبدالمولى **Age:** 46 Years **Gender:** Female
Ref. Prof. Dr.: أ.د/ مايكل فكرى **Date:** 4/8/2026

Markers: For a breast mass: Estrogen & Progesterone, Her-2 [c-erb-B2 oncoprotein] & Ki 67 receptors.

Procedure:

Sections from paraffin blocks were treated with antigen retrieval solution and then with monoclonal antibodies against Estrogen, Progesterone, Her-2 [c-erb-B2 oncoprotein] & Ki 67 receptors, using streptavidin biotin peroxidase system. DAB was used as substrate as well as chromogen. Hematoxylin was used as counter stain.

Results:

- *) Examination of sections treated for Estrogen receptors revealed strong positive nuclear staining of tumour cells [Protein Score 5/5, Intensity Score 3/3 & Total Score 8/8].
- *) Examination of sections treated for Progesterone receptors revealed strong positive nuclear staining of tumour cells [Protein Score 5/5, Intensity Score 3/3 & Total Score 8/8].
- *) Examination of sections treated for Her-2 receptors revealed strong complete membranous stain of more than 90% of the tumour cells by chromogen.
- *) Examination of sections treated for Ki 67 receptors revealed strong positive nuclear staining reactivity of more than 70% of the tumour cells by chromogen.

N.B.: Normal mammary ductal &/or lobular epithelial cells are used as a positive internal control.

Diagnosis:

***) RIGHT BREAST CARCINOMA:**

- **STRONG POSITIVE FOR ESTROGEN RECEPTORS [8/8].**
- **STRONG POSITIVE FOR PROGESTERONE RECEPTORS [8/8].**
- **STRONG POSITIVE FOR HER-2 RECEPTORS [SCORE 3; PROMINENT OVER-EXPRESSION].**
- **STRONG POSITIVE FOR KI-67 RECEPTORS [OF MORE THAN 70% OF THE TUMOUR CELLS].**




Yours Sincerely
Dr. Essam E. Ayad

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Essam Ayad**

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زميل جامعة بادوفا بايطاليا

PATHOLOGY REPORT

Patient Name: فاطمه خليفه زهران عبدالمولى **Age:** 46 Years **Gender:** Female
Ref. Prof. Dr.: أ.د/ مايكل فكرى **Date:** 4/8/2026

Clinical Data: Right Breast Mass, Lumpectomy & Axillary LNs Dissection.

Gross Pathology:

An irregular piece of fibrofatty tissue, measured 10x9x8cm. Serial sectioning revealed an ill-defined mass, greyish white in colour, firm in consistency, measured 34x32x25mm. The rest of the lump showed ill-defined greyish white streaks. The surgical margins were free of tumoural invasion with a 2cm minimal safety margin.

Dissection of the axilla revealed 35 lymph nodes, ranged in size from 4x4mm up to 10x9mm. Cut sections were greyish pink, few nodes showed greyish white cut surfaces.

Microscopic Examination: of the specimen received revealed:

Breast tissue showing marked proliferation of the breast ducts (adenosis) with scattered foci of apocrine metaplasia and variable grades of atypical epitheliosis.

Examination of the sections prepared from the mass revealed that the epithelial lining of the mammary ducts showed cellular crowding, marked cellular and nuclear pleomorphism, loss of polarity and scattered mitoses. The malignant cells were arranged in the form of distended ducts or in the form of epithelial sheets, surrounded by moderate fibrosis.

Occasional foci of intraductal component, less than 1% of the tumour tissue detected (Comedo Pattern).

No evident TILs was detected.

Some lymphovascular emboli could be seen.

No perineural invasion could be detected

Minimal tumoural necrosis was detected.

All surgical margins were free of tumoural invasion.

Examination of the Lymph nodes revealed focal metastatic malignant deposits in eight out of the 35 dissected LNs, the remaining nodes showed sinus histiocytosis. **[L.N. 8/35]**



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زميل جامعة بادوفا بايطاليا

PATHOLOGY REPORT

DIAGNOSIS:

***) RIGHT BREAST MASS, LUMPECTOMY & AXILLARY LNS DISSECTION:**

- **INVASIVE DUCT CARCINOMA OF THE BREAST, (GRADE II)**
- **OCCASIONAL FOCI OF INTRADUCTAL COMPONENT, LESS THAN 1% OF THE TUMOUR TISSUE DETECTED (COMEDO PATTERN).**
- **NO EVIDENT TILS WAS DETECTED.**
- **SOME LYMPHOVASCULAR EMBOLI.**
- **NO PERINEURAL INVASION.**
- **MINIMAL TUMOURAL NECROSIS.**
- **ALL SURGICAL MARGINS WERE FREE OF TUMOURAL INVASION.**
- **LYMPH NODES; [L.N. 8/35]**
- **STAGE: pT2 pN2 MX**

PATHOLOGY REPORT

Patient Name: فاطمة خازنة زهران محمد المولى **Age:** 46 Years **Gender:** Female

Ref. Prof. Dr.: د. محمد فاروق **Date:** 4/8/2026

Stain(s): For a breast mass. Estrogen & Progesterone, Her-2 [c-erbB2 oncoprotein] & Ki 67 receptors.

Procedure:

Sections from paraffin blocks were treated with antigen retrieval solution and then with immunohistochemical antibodies against Estrogen, Progesterone, Her-2 [c-erbB2 oncoprotein] & Ki 67 receptors, using streptavidin biotin peroxidase system. DAB was used as substrate in all immunohistochemistry. Hematoxylin was used as counter stain.

Results:

Immunohistochemical staining revealed positive nuclear staining of tumour cells for Estrogen, Progesterone, Her-2 [c-erbB2 oncoprotein] & Ki 67 receptors. The results are as follows: Estrogen (3+/3), Progesterone (3+/3), Her-2 [c-erbB2 oncoprotein] (2+/3) & Ki 67 (3+/3).
Examination of sections stained for Her-2 revealed strong complete membranous stain of more than 90% of the tumour cells by chromogen.

Ayad Lab



Fatma Kh.

04082026

Yours Sincerely

Dr. Essam E. Ayad



S C A N

Name: Fatma Khalifa Zahran
Referred by: Prof.

ID: 5237744
Date: 18/07/2026

ABDOMINAL AND PELVIC ULTRASONOGRAPHY

- The liver is mildly enlarged in size measuring 17 cm, showing bright homogenous parenchyma with no focal hepatic lesions.
- No dilated biliary radicles.
- The portal vein and common bile duct are within normal limits.
- The gall bladder is surgically removed.
- Midline is masked by gases.
- The spleen is of average size (11.5 cm) with homogenous echopattern.
- Both kidneys are within normal size, shape and echo-pattern with preserved differentiation and no back pressure changes or detected renal stones.
- The urinary bladder shows average capacity with normal wall thickness. No stones nor masses seen inside.
- The uterus is anteverted, anteflexed of normal size, shape and echopattern with no focal lesions. IUD is seen insitu.
- No gross adnexal cystic or solid mass lesions.
- No pelvic masses or collection.
- No ascites.

IMPRESSION:-

- Mild bright hepatomegaly likely fatty for clinical and lab correlation.
- Colonic gaseous distension.

MUCH OBLIGED
Ahmed Emad, MD

Name: Fatma Khalifa Zahran
Referred by: Prof.

ID: 5237744
Date: 18/07/2026

High Field Open (1.5T) Dynamic MR-Mammography of Both Breasts

Technique :

- Axial T2 (plate 1).
- Axial STIR (plate 2).
- Sagittal postcontrast 3D TFE of the right breast (plate 3).
- Sagittal postcontrast 3D TFE of the left breast (plate 4).
- Dynamic multiphase post-contrast study done in 8 minutes with MIP reconstruction (plate 5).
- Time intensity curves (plate 6).

Findings:

- Bilateral glandular condensation and evidences of bilateral homogenous glandular enhancement.
- Right breast UOQ 10:00 welldefined micronodular outline mass with coma tail ductal extension, it measures 30x31 mm in diameters, the lesion is low signal in T2 and STAIR with intense mass like enhancement. The kinetic data obtained indicates a malignant pattern, rapid rise followed by wash out and plateau, type 2& 3 curves. A small nearby one 8x10 mm is seen at the lesion upper outer aspect, it shows similar morphology & kinetic data
- No left breast masses and no architecture distortion.
- Normal skin thickness and contour of both breasts.
- Enlarged level 1 right axillary lymph nodes the largest 12x10 mm in diameters
- There is multiple likely non-specific left axillary lymphadenopathy.

OPINION :

Right breast UOQ multifocal malignant masses, categorized as BIRADS 5 based on the morphological and the kientic data. Microbiopsy was done Normal study of the left breast, BIRADS 1.

Prof. Naglaa Abdel Razek.MD

Defining BIRADS classification of breast lesions according to the American Collage of Radiology:

<u>Category</u>	<u>Suspected Cancer Risk</u>
BIRADS 0 : Non-informative study.	0 %
BIRADS 1 : Normal study.	0 %
BIRADS 2 : Benign findings.	2 %
BIRADS 3 : Probably benign.	50 %
BIRADS 4 : Probably malignant.	
A : mild suspicion.	
B : moderate suspicion.	
C : high suspicion.	98-100%
BIRADS 5 : Malignant.	
BIRADS 6 : Pathologically proven malignant lesion.	



S C A N

Name: Fatma Khalifa Zahran
Referred by: Prof.

ID: 5237744

Date: 08/07/2026

Thanks for reference

Bilateral digital mammography , high resolution breast sonography & Elastography revealed :

- Scattered mixed densities of glandular condensation are seen involving both breasts.
- Right breast UOQ asymmetric density with US detected illdefined solid hypoechoic mass in the 10:00 location , hard by elastograpy . it measures 3x3 cm in diameters and a smaller similar lesion is seen at its upper aspect , it measures 8x10 mm in diameters
- No left speculated masses or pathological microcalcification.
- Normal skin thickness & contour of both breasts.
- No left breast solid or cystic masses.
- Enlarged right axillary lymph nodes , level 1 the largest 10x12 mm in diameters
- Small sized likely non-specific left axillary lymph nodes.

OPINION :

Bilateral mild mammary fibroadenosis , left breast , BIRADS 2.

Right breast UOQ highly suspicious multifocal masses , BIRADS 4C , biopsy is recommended.

Kindly accept my best regards,
Dr. Naglaa Abdel Razek, M.D.

Defining BIRADS classification of breast lesions according to the American Collage of Radiology:

Category	Suspected Cancer Risk
BIRADS 0 : Non-informative study.	0 %
BIRADS 1 : Normal study.	0 %
BIRADS 2 : Benign findings.	2 %
BIRADS 3 : Probably benign.	50 %
BIRADS 4 : Probably malignant.	
A : mild suspicion.	
B : moderate suspicion.	
C : high suspicion.	
BIRADS 5 : Malignant.	98-100%
BIRADS 6 : Pathologically proven malignant lesion.	



LABS
Collection Date: 19/07/2026
Review Date: 22/07/2026

Patient Name: الأستاذة/ فاطمة خليفة زهران
Patient No.: 1-383151
Accession No.: 1260742821
Age: 46Y

Referring Doctor: أ.د. نجلاء عبدالرازق

PATHOLOGY UNIT

Path. No.: (10602-26).

Clinical Data & Nature of the specimen:

Right breast UOQ mass, Tru-Cut biopsy.

Gross Examination:

Four grayish white tissue cores, measured up to 1.3 cm long, totally submitted.

Grossing by S. Ahmed

Microscopic Examination:

Serial sections examined revealed cores of breast tissue with wide areas of fibro-hyalinosis and lobular adenosis with a tiny focus of atypical ductal proliferation exhibiting nuclear enlargement and hyperchromasia.

Diagnosis:

Right breast UOQ mass, Tru-Cut biopsy:-

*Minimally represented, highly suspicious focus of atypical ductal epithelial proliferation (<1 mm).

Comment:-

*The suspicious lesion is very tiny and not sufficient for IPT studies.

*Move lesional tissue biopsy is required for re-evaluation and IPT studies.

Prof. Dr. Wael Shawky, MD

Prof. Dr. Samia Gabal, MD

Reviewed By:



Pt. Branch: وحدات الرئيسية

Page 1 of 1

Printed by: 39048 علاء سعود عبدالرحيم

AMG
ALFA MEDICAL GROUP

A MEMBER OF ALFA MEDICAL GROUP

عامل الفا

1619

Pathology Report

[[DOCTYPE:PATHOLOGY_REPORT]]



S C A N

Name: *Fatma Khalifa Zahran*

ID: 5237744

Date: 18/07/2026

REPORT

Digital radiographic examination of the chest (PA View) revealed:

- Unremarkable both lung fields and costophrenic angles.
- Normal cardiothoracic ratio.

Dr. Mohamed Raafat, MD

د. يوسف
رؤس
9-27



مبادرة رئيس الجمهورية
لدعم صحة المرأة المصرية



09/27/2026

MDT FORM

Name:	الطه خليه ... زهران عبدالولي		
Age:	47 Y	MRN:	13134
Menopausal Sta	PRE	ID:	28001102104244
NRS SCORE:		Phone:*	01099928266
BMI:		Marital Status:	M
Gender		Family History:	-VE
CO Morbidity:	NAD	Reason Of MDT:	
Surgical History:*	POST RT CBS + ALND		
ECOG Performance Status	☐ 0 ☐ 1 ☐ 2 ☐ 3 ☐ 4		

Clinical Data:

BX:	IDC GII
ER: 8/8	PR: 8/8
HER2: 3+	KI67: 80

History:

RT CBS +ALND (8/35)
ALL SURGICAL MARGINS ARE FREE
T2 N2 M0

Sonomammography:

DYNAMIC MRI

NO RESIDUAL LESIONS

Biomarker/Molecular

Mets Workup:

BONE SCAN FREE

CT CAP FREE

Recommendations:

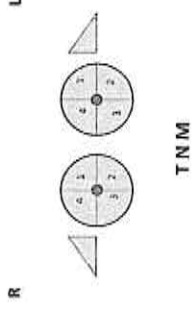
NO NACT TAKEN PT UNDERWENT UPFRONT SURGERY FIRST

MDT Outcome & Plan

for adj AC* 4 CYCLES THEN TAXOL WEEKLY 12 WEEKS WITH HERCEPTIN
FOR 1 YEAR

PORT

ADJ TAM/ZOLADEX 2 YEARS THEN SHIFT TO AI/ZOLADEX



TNM

Right Side

Left Side

Q1	Q2	Q3	Q4	Q1	Q2	Q3	Q4

Clinical Staging:

Pathological Staging:

SIGNATURE

MEDICAL ONCOLOGY	SURGERY	RADIOLOGY	PATHOLOGY
د. حنان عورت	د. احمد لطيف		

about:blank